Authrex Medical Associates

Provider-Side Prior Authorization · Oncology
1 Care Square, Suite 400 · San Francisco, CA 94105
Tel: (415) 555-0140 · Fax: (415) 555-0141 · NPI: 1234567890

May 6, 2026

Aetna Health Inc.
Attn: Medical Director · Oncology Appeals
P.O. Box 14079
Lexington, KY 40512

Re: Prior Authorization Appeal — Patient J.D. — trastuzumab (J9355)

Patient initials:	J.D.
Treatment requested:	trastuzumab (J9355)
Original denial date:	2026-05-04
Payer:	AETNA

Dear Medical Director,

This letter constitutes a formal appeal of your decision dated May 4, 2026 to deny coverage for trastuzumab (J9355) in this 57-year-old female patient with stage IIIA HER2-positive invasive ductal carcinoma of the right breast (ICD-10 C50.911). We respectfully request that your decision be overturned and that prior authorization be granted under Aetna CPB 0084 § II.B Initial Authorization Criteria.

Clinical background: 57-year-old patient diagnosed August 2025 with HER2-positive (IHC 3+, FISH amplified ratio 6.4) stage IIIA invasive ductal carcinoma. The patient has completed standard adjuvant chemotherapy (AC-T regimen, four cycles, partial response on docetaxel) and is now eligible for HER2-targeted maintenance therapy. Baseline cardiac assessment shows LVEF 62% by transthoracic echocardiogram on April 15, 2026, well within the 60-day window required by both the trastuzumab FDA label (§ 5.1) and Aetna policy. Performance status is ECOG 1.

Argument summary: The denial appears to have been based on an absent documentation flag for HER2 confirmation testing, but pathology confirms HER2 IHC 3+ AND FISH amplification — both reflexively reported by Quest Diagnostics on September 5, 2025. We have attached the corresponding pathology report for the medical director's review.

We respectfully request expedited reconsideration given the time-sensitivity of HER2-targeted adjuvant therapy initiation.

Structured arguments — point-by-point response

Argument 1 — HER2-positivity (Aetna CPB 0084 § II.B.1)

Payer position

Denial cited insufficient documentation of HER2-positive status.

Authrex counter-position

HER2-positivity is unambiguously documented by both IHC (3+) and reflex FISH (amplified, ratio 6.4) on the same specimen, satisfying both ASCO/CAP 2018 testing guidelines and Aetna § II.B.1.

Cited clinical evidence

- HER2 IHC 3+ on Observation obs-her2 (test_date 2025-09-05)
- HER2 FISH ratio 6.4 (amplified) on Observation obs-her2-fish (test_date 2025-09-05)
- Quest Diagnostics pathology report dated 2025-09-05 (attachment 1)

Cited policy excerpt

Coverage requires documented HER2-positivity by IHC 3+ OR ISH amplified, per Aetna CPB 0084 v2024.3 (rev 2024-08) § II.B.1.

Cited guideline

NCCN Guidelines Breast Cancer v.4.2024 BINV-K — adjuvant trastuzumab is standard of care for HER2-positive stage I-III breast cancer.

Argument 2 — Baseline LVEF assessment (Aetna § II.B.3)

Payer position

(Inferred) Documentation of cardiac assessment may not have been considered in the original review.

Authrex counter-position

Baseline transthoracic echocardiogram on April 15, 2026 documents LVEF 62% — comfortably above the ≥ 50% threshold and well within the 60-day pre-initiation window required by both the FDA label and Aetna policy.

Cited clinical evidence

- LVEF 62% on Observation obs-lvef (test_date 2026-04-15)
- Echocardiogram report (attachment 2)

Cited policy excerpt

Baseline LVEF ≥ 50% by ECHO or MUGA within 60 days of initiation, per Aetna CPB 0084 § II.B.3.

Cited guideline

Trastuzumab HCP § 5.1 Cardiomyopathy — baseline LVEF assessment required prior to initiation; reassessment every 3 months on therapy.

Requested action: Overturn the denial and authorize trastuzumab (J9355) for adjuvant HER2-targeted therapy at standard dosing per the trastuzumab FDA label and NCCN BINV-K.

Respectfully submitted,

Authrex Clinical Authorization Team

On behalf of the requesting provider

Attachments referenced (5)

- Quest Diagnostics pathology report dated 2025-09-05 (HER2 IHC + FISH)
- Echocardiogram report dated 2026-04-15 (LVEF 62%)
- AC-T treatment summary (cycles 1–4, completed 2026-01-15)
- ECOG performance status documentation dated 2026-04-20
- ASCO/CAP HER2 Testing Guideline 2018 (reference document)

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